Independent coverage evaluation survey of the National Deworming Day round

Training of nodal officers and master trainers, Madhya Pradesh, November 2026

Prepared by the WHO Country Office for India for NHM Madhya Pradesh and AIIMS Bhopal

Training led by AIIMS Bhopal, online: 4 November 2026 (nodal officers and survey teams)

Version 1.3, 3 November 2026

Drafted with the assistance of Claude (Anthropic); reviewed by the AIIMS Bhopal survey team.

What this training covers

By the end of the training, participants can:

1. Explain why reported NDD coverage is checked with a household survey
2. Make segments and select households exactly as planned (SOPs 2–3)
3. Complete the household form, F4 and F10 on the device
4. Verify school and AWC records and calculate verification factors (SOP 9)
5. Supervise teams, back-check households and correct problems (SOP 8)
6. Train the survey teams, using the field manuals (SOP 7)

[dd Nov] Classroom	Design, sampling, forms, digital form, record verification, quality assurance
[dd Nov] Field day	Practice and pretest in a village and an urban ward outside the sample
[dd Nov] Tools final	Changes from field testing made to the forms (v0.6, F4 v1.1, F10 v1.2)
4 Nov All teams	Online training of survey teams and nodal officers, led by AIIMS with the master trainers

Basis: Technical support package, SOP 7 and table 10; indicative State timeline (annex B).

Why reported coverage needs an independent check

Reported coverage = children ticked as dewormed ÷ children targeted (enrolled, registered or listed). **Children never listed are in neither number,** and errors in tallying and compiling go unseen. Madhya Pradesh has reported more than 95% coverage for several rounds; a household survey shows whether that is real.

0.57

verification factor for enrolled children in the 2016 NDD round in Uttar Pradesh: school coverage 47–67%, against 82% reported

52%

of schools in that exercise kept a copy of the reporting form, so most figures could not be checked at source

19 of 77

surveyed mass drug administrations in Asia had reported coverage within ±10 points of surveyed coverage

What the survey adds: a statistical estimate of how many children actually swallowed the tablet, including out-of-school and unregistered children, and a test of whether the reported figure lies within its 95% confidence interval.

Sources: Evidence Action (2016); Zoerhoff et al. (2023); WHO field manual (2019), module 1; State letter of 16 September 2026.

The survey at a glance

4

survey districts drawn at random, plus 2 reserves

30

clusters per district: 10 in each block, by probability proportionate to estimated size

≈38

households selected in each segment of about 50 households

≈1,140

households and about 1,700 children aged 1–19 years per district

3

blocks per district, drawn with probability proportionate to population, to keep travel within reach

2

record checks per cluster: the school and the AWC, plus block and district

Basis: Approved methodology (annex A), adapted to two-stage cluster selection; technical support package, table 1.

Who is in the survey

Reference period: 6 October (NDD) and 9 October (mop-up day) 2026, the “deworming days”

Children aged 1–19 years

Form F6

- Usual residents of the household during 6–9 October
- Enrolled or not; registered at an AWC or not
- Include those absent today: revisit, phone or proxy
- Parent or caregiver answers for children under 10

Not in this survey

Not listed

- Women aged 20–49 years: excluded; monitoring data already show low coverage
- Children under 1 year: not targeted by NDD
- Visitors who arrived after 9 October
- Adults aged 20 years and above: counted in H5a only

Listing rule: a child who lived here during 6–9 October but has since moved away is still listed, with a household member as proxy. A visitor who arrived after 9 October is not listed.

Timeline and roles

8 Oct
District draw

9–11 Oct
Sampling frames

2 Nov
Cluster selection

[dd Nov]
Nodal officer training

4 Nov
Team training (online)

[dd–dd Nov]
Household survey, 2 phases

[dd Nov]
Pending clusters, back-checks

[dd Dec]
Preliminary findings

No fieldwork during Diwali (7–11 November). Dates to be confirmed by NHM MP and AIIMS; joint analysis by AIIMS and WHO after field work.

NHM Madhya Pradesh

Lead and approvals; engages the State survey coordinator; frames; reported NDD data; letters; independent monitors and their budget

State survey coordinator

Independent, engaged by NHM: facilitates the draw; checks frames and the block and cluster selection; co-facilitates training; coordinates monitors

AIIMS Bhopal

Conducts the draw and the block and cluster selection (selector tool); tools and digital form; leads the trainings and pretest; data management; report

Government Medical Colleges

Community Medicine teams in pairs: segmentation, household selection, interviews and record verification; nodal officers supervise

WHO, New Delhi

Remote support: guidance and tools; review of sampling and tools; online training if needed; weekly data review; joint analysis

Basis: Indicative State timeline (annex B); roles adapted from annex C; technical support package, tables 2–3.

Field manuals: one for each stakeholder

	For	Covers	Language
1	Survey teams (pairs)	Segmenting, List A/B, household form, F4, F10, sending data	English, Hindi scripts
2	Nodal officers, SRs	Pairs, schedule, F9, F8, F4 sign-off, daily report, escalation	English
3	DIO, BMO, BEO, CDPO; partners	Purpose, support needed, independence rules, timeline	English
4	ASHA, AWW, headteachers	Why the team comes, how to help, what not to do	Hindi
5	AIIMS data manager	Server, forms, QR codes, phase release, daily checks, backups	English
T	Master trainers	This deck, with manuals 1–2 as handouts	English

Sources: NDD CES MP field manuals 1–5 (AIIMS Bhopal, November 2026); nomination list of 2 November 2026.

Sampling in stages

District

4 of the State's districts, drawn at random in a witnessed session

SOP 1

Block → cluster

3 blocks per district, then 10 villages or wards per block, both by PPS (population)

SOP 2

Segment

Planned number of segments of about 50 households; one drawn at random

SOP 3

Household

Every 1.32nd household on List A or B: about 38 per segment

SOP 3

Persons

Every child aged 1–19 years who lived in the household during 6–9 October

SOP 4

Weights: within a block, larger subunits are more likely to be selected but are divided into more segments, and only one is surveyed. Blocks are drawn by population, so each household's weight comes from the block and cluster probabilities in the selector tool; segments must still be made exactly as planned.

Sources: WHO field manual (2019), sections 1.5 and 2.1–2.2, annex 2; AIIMS cluster selector tool.

Stage 1: the witnessed district draw

Random
Every district reporting NDD coverage has the same chance

Witnessed
At least three witnesses from different organizations

Documented
Every number and its order recorded on form F1

Confidential
Names shared only with signatories until letters are issued

On 8 October (SOP 1)

1. Number the official list in alphabetical order (55 districts in table 5; use the official list if it differs)
2. Witnesses check one folded slip per number; mix the slips in a bowl
3. A witness draws a slip and reads it aloud; record it on F1; do not return it
4. Draws 1–4 are the survey districts; draws 5–6 are reserves 1 and 2
5. All witnesses sign F1; enter the numbers in the Draw sheet and check the names

Reserves are used only for a documented reason that prevents fieldwork (for example, a disaster or a security restriction), in draw order and agreed by NHM, AIIMS and the State survey coordinator. Low reported coverage or distance is never a reason.

Sources: Technical support package, SOP 1 and form F1; WHO field manual (2019), table 1.1.

How the sample size was set

$$n = DEFF \times z^2 \times p(1-p) \div [\delta^2 \times (1-r)] = 4 \times 1.96^2 \times 0.5 \times 0.5 \div (0.05^2 \times 0.9)$$

= 1,707 children per district

p = 50%

expected coverage: conservative, as true coverage is unknown

δ = ±5 points

precision: half-width of the 95% confidence interval

DEFF = 4

design effect for cluster sampling

r = 10%

non-response

1,707
children per district

÷ 1.5
children aged 1–19 years per household

= 1,138
households per district

÷ 30
clusters

≈ 38
households per segment

of 50
households in a segment: every 1.32nd

With p = 80% (reported coverage minus 15 points) the sample would fall to 1,093 children, but precision would be poor if true coverage were much lower, which is what the survey is designed to detect.

Sources: WHO field manual (2019), box 1.1 and annex 2; NFHS-5 Madhya Pradesh (mean household size 4.6); workbook, Sample_size sheet.

Building the sampling frame

- **One line per subunit:** inhabited villages and the wards of statutory towns; each census town is one subunit
- **Every household in one subunit only:** no gaps, no overlaps
- **Counts:** latest State source, or Census 2011 projected to 2026
- **Order:** by block, then census location code, never alphabetical
- **Merge** subunits with fewer than 25 households with the next adjoining one
- **Split** subunits with more than 400 households where possible (recommended, not required)

Line	Block	Subunit	House-holds	Note
1	Block A	Village 01	120	
2	Block A	Village 12 and Village 13	118	Village 13 had 18 households: merged
3	Block A	Census town 1	380	One subunit; no wards
4	Block B	Ward 12, part 1 of 3	420	Ward of 1,260 households split
5	Block B	Ward 12, part 2 of 3	420	
6	Block B	Ward 12, part 3 of 3	420	

Who: the NHM data cell prepares the frame with AIIMS by 11 October; the State survey coordinator checks it with form F2 before the witnessed selection on 12 October.

Sources: WHO field manual (2019), box 1.2; technical support package, SOP 2 and form F2.

PPES selection: a worked example

Line	Subunit	House-holds	Seg-ments	Cumul-ative	Point
1	Village 01	120	2	2	
2	Village 02	180	4	6	3.52
3	Village 03	400	8	14	13.19
4	Village 04	90	2	16	
5	Village 05	150	3	19	
6	Village 06	260	5	24	22.85
7	Village 07	75	2	26	
8	Village 08	110	2	28	
9	Village 09	310	6	34	32.52

N = 290
segments in the whole (fictional) district

Interval = 290 ÷ 30 = 9.67
one selection every 9.67 segments

Start = 0.3642 × 9.67 = 3.52
random number 3642 from the table

Points: 3.52, 13.19, 22.85, 32.52 …
each point selects the line whose cumulative total first reaches it

A large subunit can be selected twice: Ward 5 (1,000 households, 20 segments) gets two points, so the team draws two different segments there.

Sources: WHO field manual (2019), box 1.2 and annex 2; workbook, Training sheet (fictional frame of 65 lines).

Segmentation in the field

1. **Take the planned number of segments shown in F4 (2011 households ÷ 50). Never change it.**

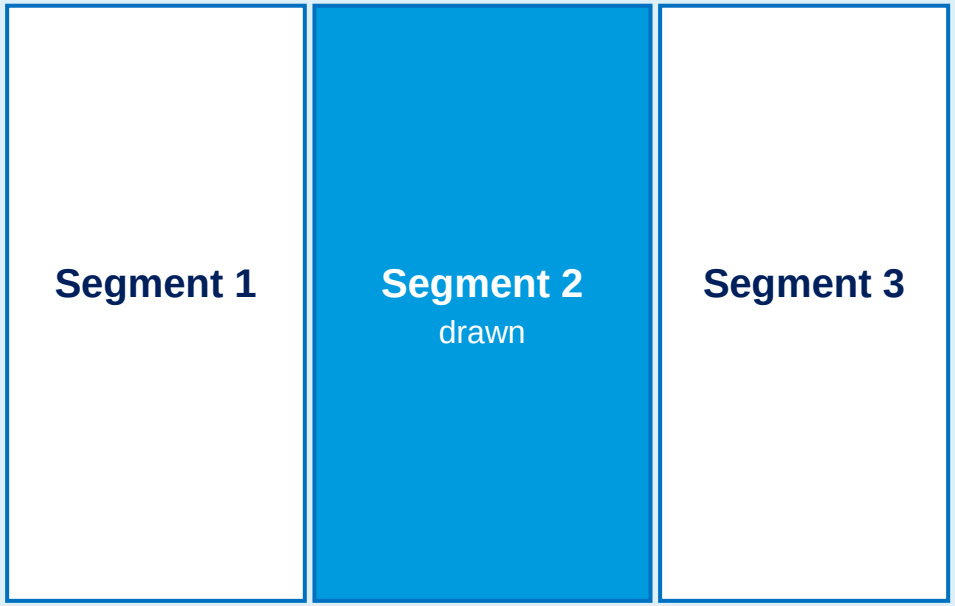
2. **Divide the subunit with the local guide** into that many parts with similar numbers of households, along roads, paths or streams.

3. **Draw a sketch map** on F4 and number the segments.

4. **Draw one segment** with numbered slips, in front of the guide; take a GPS point at its centre.

Selected twice? Draw two different segments.

Large unit (over 600 households; 24 clusters)? F4 gives the number of parts: divide along mohallas, draw one part, estimate its households, then segment that part.



Example from the WHO manual

Planned: 131 households → 3 segments.

Found: 106 households → still 3 segments, of about 35 households each.

Sources: WHO field manual (2019), box 1.3 and section 2.1; technical support package, SOP 3 and form F4.

Selecting households with List A or List B

Toss a coin

Heads: List A. Tails: List B. Record it on F4.

Agree the route

With the guide, a route past every household, and the first one.

Number every household

Chalk numbers on doors; count households, not buildings.

Stop at listed numbers

Interview all eligible persons there; go on to the end of the segment.

This survey's List A (random number 1706, interval 1.3181): about 3 households in 4

1	**2**	**3**	4	**5**	**6**	**7**	8	**9**	**10**	**11**	12	**13**	**14**	**15**	16	**17**	**18**	**19**	**20**

Filled: households interviewed (1, 2, 3, 5, 6, 7, 9, 10, 11, 13 …). White: counted and passed (4, 8, 12, 16). List B (8069) starts 2, 3, 4, 6 …

No replacement. An absent or refusing household is not replaced by a neighbour: revisit later the same day and record its result code. Do not stop when a number is reached, and do not add households beyond the end of the segment.

Sources: WHO field manual (2019), box 1.4; sampling workbook v1.1 (HH_lists); List A/B cards, 3 November 2026.

Counting, listing and who answers

#	First name	Sex	Age	Present	In the survey
–	Ravi	M	41	Y	Counted in H5a only
–	Sunita	F	36	N	Counted in H5a only
1	Meena	F	19	N	C: F6 by proxy
2	Arjun	M	12	Y	C: form F6
3	Priya	F	7	Y	C: form F6

H5a counts all 5 usual residents during 6–9 October, absent or not; H5b counts the 3 aged 1–19 years, and only they are listed by name, sex and age. Meena moved to Indore after the round: she is still listed and her father answers.

Ask H6–H7 (awareness) before the person forms, so that answers about the tablet do not influence what the household says it heard beforehand.

1–9 years
A parent or caregiver answers

10–17 years
The adolescent answers, with a parent's consent and his or her own assent

18 years and above
The person answers and gives consent

Absent
Revisit the same day, then telephone; then an adult household member answers as proxy

Sources: WHO field manual (2019), section 2.3; technical support package, SOPs 3–5.

The household form at a glance

F5

Household form

One per selected household

- H1 identification: cluster, segment, list, household number, team, interviewer, sampler, language, GPS
- H2 result of visit (codes 1–7)
- H3 consent; H4 respondent
- H5 counts (residents; children 1–19) and listing of children
- H6–H7 awareness; H8 summary

Ask H6–H7 before the person forms

F6

Child form

One column per child aged 1–19 years

- C1–C1a who answers; assent
- C2–C3 enrolment, class, AWC registration
- C5–C6 offered; swallowed
- C7–C9 where, which day, supervised
- C11–C12 reasons; C13 problems after

Coverage counts only C6 = yes

v0.7

Form v0.7 changes

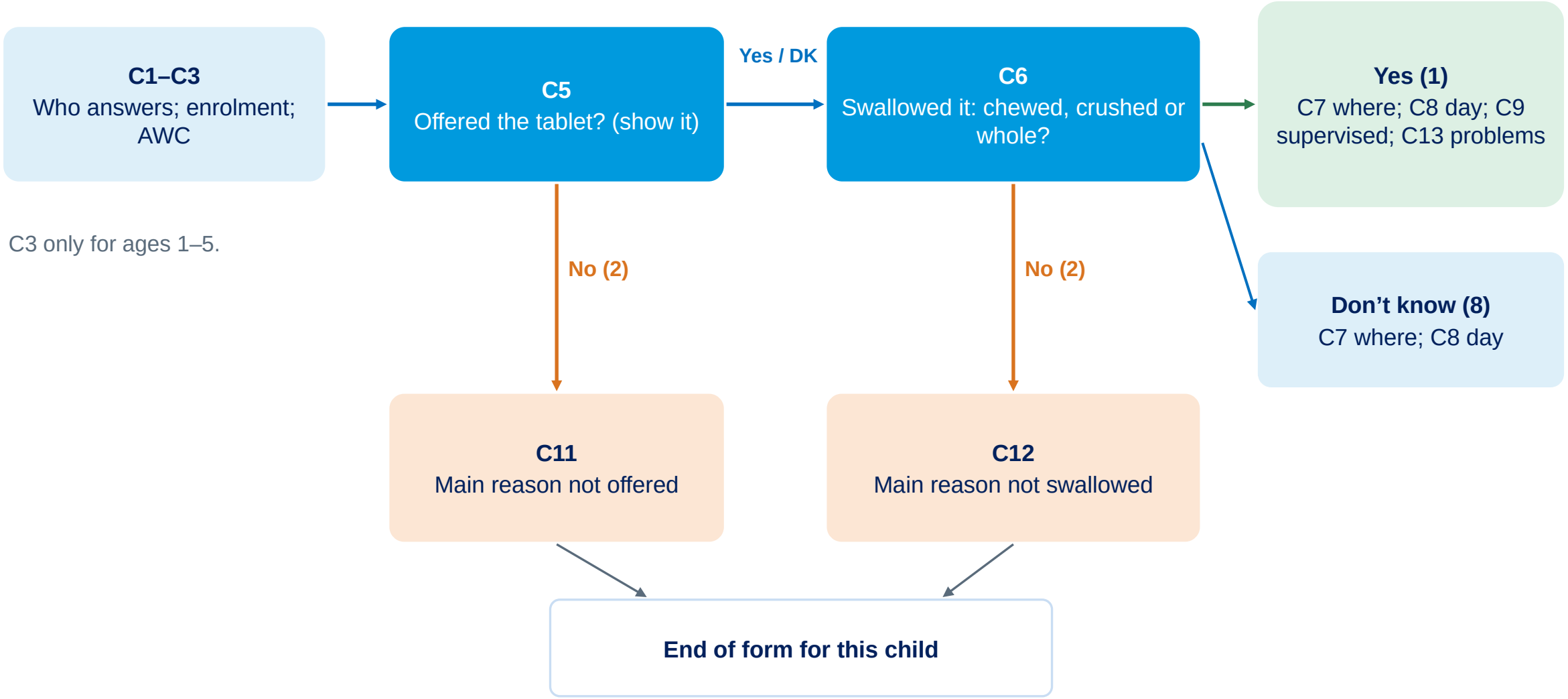
Paper F5–F6 are the back-up only

- District → block → cluster chosen from the selected list
- Household number checked against List A or B
- Each child section starts with name, age and who answers
- “No one present can answer” only after a revisit or call
- Listing must match the H5b count

Check the pre-filled identification at every household

Sources: Technical support package, SOP 5 and forms F5–F6 (adapted from WHO field manual, annex 1, appendix 1); XLSForm v0.7.

Child form: the skip pattern



C3 only for ages 1–5.

Sources: Technical support package, form F6; the digital form applies the same skips automatically.

Interviewing well

Do

- Read the consent statement in full; obtain assent from adolescents
- Show the sample tablet and describe the deworming days: dates, school or anganwadi, who gave it
- Read each question as written, in the respondent's language
- Record exactly what the respondent says; "don't know" is a valid answer
- Thank the household; do not discuss results

Do not

- Suggest an answer ("Surely he took it at school?")
- Interview with a teacher, AWW or ASHA present
- Leave out people who are absent today
- Accept a neighbour as proxy
- Change the list, the segment or a household

Recall is reliable when prompted well: in Togo, survey answers on swallowing MDA medicines agreed with treatment registers for more than 95% of people at one month. Interviews here take place 10 days to six weeks after the round.

Sources: WHO field manual (2019), annex 3; Budge et al. (2016).

The digital form

Works offline

ODK Collect on Android; send data when connected

GPS and time stamps

Point at the door; start and end times recorded automatically

IDs built in

Household and person IDs, for example 434-098-0810-024-02

Skips and checks

Household number must be on List A or B; impossible ages blocked

Back-check flag

About 10% of completed households flagged at random

Team details

Team number matches the app user (ratlam01 = team 1); interviewer and language recorded

Every evening: send all forms, charge devices and report problems to the supervisor.

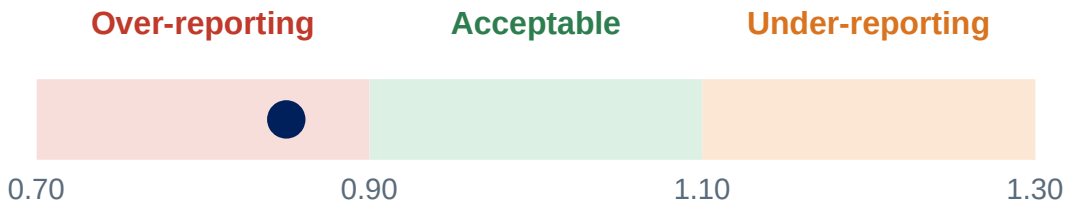
Paper forms F5–F6 are the back-up if a device fails. Never edit a sent form: the data manager makes corrections, with a log.

Sources: XLSForm v0.7 (ODK Validate passed) on the survey server odk.cesndd.in; technical support package, SOP 6 and table 9.

Record verification and the verification factor

VF = agreed recount ÷ reported

Example: registers show 170 children ticked; the reporting form says 200. **VF = 170 ÷ 200 = 0.85:** over-reporting.



WHO accepts a VF of 0.90–1.10 as accurate enough for NTD reporting. Ask what may explain any value outside it; do not argue or correct the records.

At the school and the AWC (form F10)

1. Ask for registers, the ASHA list and the retained reporting form
2. Note whether single (NDD) and double (mop-up) ticks were used
3. Two people recount independently until they agree
4. Copy the reported figure; calculate the VF by category
5. Record albendazole received, used and in stock

At block and district (form F11), 12–19 November

- Forms expected, received, complete and on time
- Site figures carried forward correctly; totals re-added
- Final figures compared with the copy saved on 12 October

Sources: WHO field manual (2019), module 2, section 2.3; NDD operational guidelines (2016); technical support package, SOP 9.

Reading the survey and verification together

	Reported coverage within the 95% CI	Reported coverage above the 95% CI
VF 0.90–1.10 registers match reports	**Reporting system working** Maintain; survey other districts in later rounds	**Targets too small, or children never listed** Records match reports, so check denominators and outreach to unlisted children
VF below 0.90 reports exceed registers	**Errors may cancel out** Review the sites with a low VF; retrain on tick marks	**Over-reporting in registers or compilation** Data quality assessment; retrain on recording and compiling; revise targets

Low surveyed coverage with VF in range: children were missed (absent, out of school, not registered); C11–C12 show why.

Sources: Adapted from WHO field manual (2019), table 1.4; technical support package, SOP 10 and table 15.

A supervisor’s day

Before leaving

Check each team’s kit: cluster list, Lists A and B, forms, consent statements, sample tablets, charged devices. Brief on the day’s clusters.

Uses: F4, Lists A and B

In the cluster

Observe segmentation, the segment draw and the coin toss. Check the sketch map and the household numbering.

Uses: F4, F9

During interviews

Observe at least one interview per team. Re-interview households flagged for back-check within 48 hours.

Uses: F8, F9

Evening

Check F4 against the forms; review the daily data summary; plan revisits; report problems to AIIMS and the State survey coordinator.

Uses: daily checks (table 12)

Sources: Technical support package, SOP 8, tables 11–12 and forms F8–F9.

Quality assurance: who checks what

	Who	What	How often
Team	The pair: both members segment, select and interview	Segmentation and list followed; forms complete; IDs correct; F4 completed	Every household and cluster
Supervisor	GMC nodal officer or SR, for the district's 3–5 pairs	Observe segmentation and interviews; check F4 and forms; back-check flagged households within 48 hours (F8)	Each team at least every second field day
Monitors	Independent monitors arranged and funded by NHM, outside their own districts	Observe selection, interviews and record verification (F9); re-interview 3 households in each cluster visited (F8)	At least 2 field days and 6 clusters in each district
Coordinator	Independent State survey coordinator engaged by NHM	Plans and tracks the monitors' visits; spot checks in each district; compiles F8–F9; follows up corrective action	Each district early in fieldwork; weekly note
Data	AIIMS data manager	Daily checks: duration, GPS, missing values, age heaping, proxies, list use, back-check agreement	Every evening; weekly summary to WHO
WHO	WHO Country Office, New Delhi (remote)	Reviews the weekly data quality summary, back-check agreement and monitors' findings; technical advice	Weekly review and online call

Sources: Approved methodology (annex A, quality assurance); technical support package, SOP 8 and tables 11–12.

When something goes wrong

Problem	What to do
Segments made differently from the plan	Stop. AIIMS and the State survey coordinator decide, on the supervisor's advice, whether the cluster is repeated; record the decision
A neighbour interviewed instead of the selected household	Not allowed: discard that interview, visit the selected household and record its result code
Back-check agreement below 90%	Retrain the team and back-check another 10% of its households
Cluster cannot be reached safely	Record the reason on F3 and try again by 19 November; if more than two in a district, AIIMS and the coordinator decide, after consulting WHO
Forms incomplete or IDs wrong	Revisit the same day if possible; corrections only by the data manager, with a log
Teacher, AWW or ASHA joins an interview	Politely ask to continue alone; note it on F9



Never replace a household or a cluster on your own, and never change data to make it agree.

Sources: Technical support package, SOPs 2, 3 and 8.

Indicators and how they are estimated

Indicator	Definition
Surveyed coverage (primary)	Children who swallowed the tablet ÷ children surveyed
Programme reach	Children offered the tablet ÷ children surveyed
Compliance	Children who swallowed ÷ children offered
Supervised administration	Took the tablet in front of a teacher, AWW or health worker ÷ swallowed
Awareness	Households that heard of the deworming day beforehand ÷ households interviewed
Verification factor	Agreed recount ÷ reported, by site type and level

Official estimates

Stata (svy) or the R survey package: districts as strata, blocks as primary sampling units; 95% confidence intervals and design effects

Weights

From the block and cluster selection probabilities in the selector tool

Cross-checks

Coverage Survey Analysis Tool, and the Results sheet of the workbook

Pooled estimates

Districts as strata, weighted by target population; they describe the survey districts, not the State

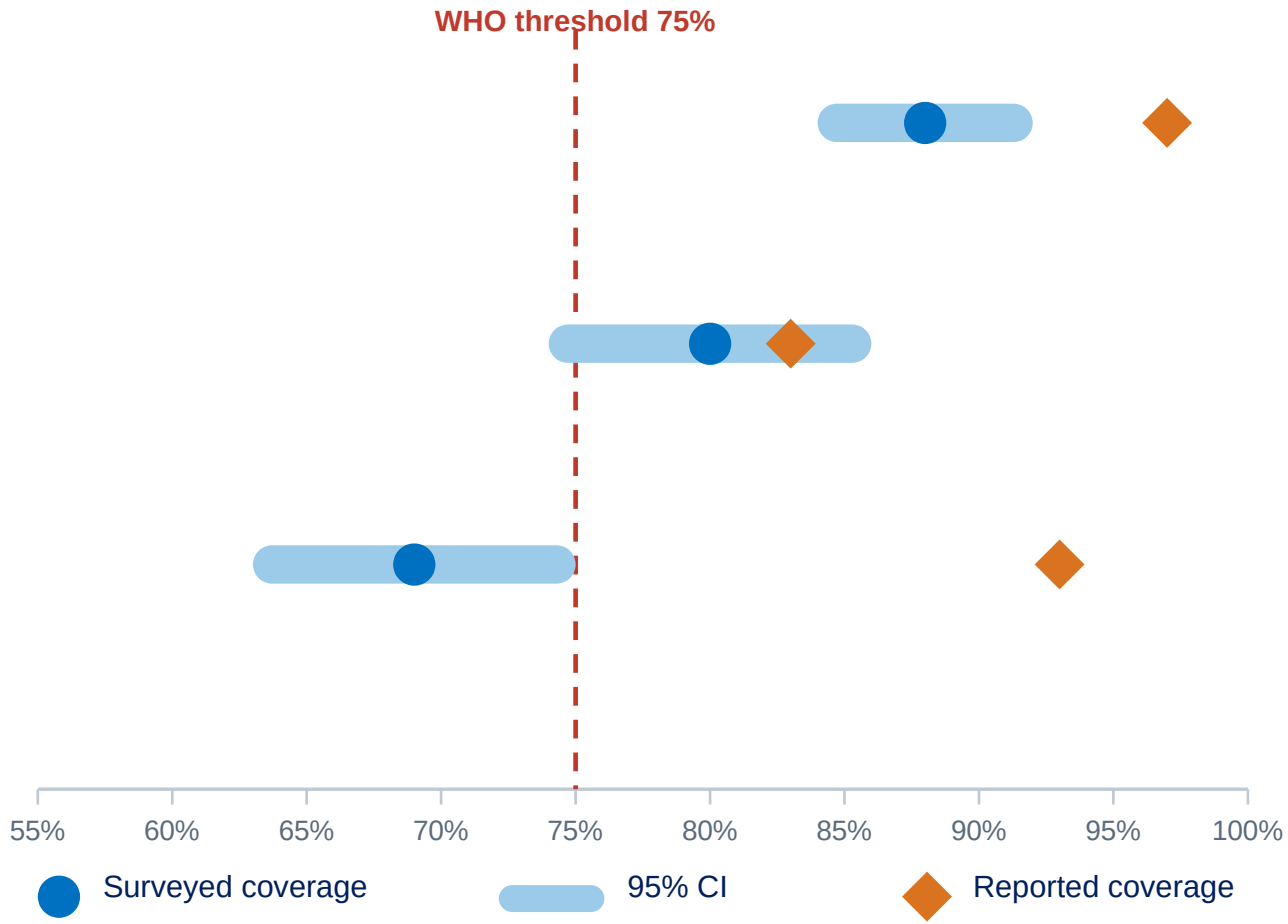
Sources: WHO field manual (2019), section 3.1; technical support package, SOP 10 and table 14.

Decision rules: validation and the 75% threshold

District A
88% (84–92%); reported 97%

District B
80% (74–86%); reported 83%

District C
69% (63–75%); reported 93%



Not validated: reported above the upper limit. Above the threshold.

Validated. Not shown to reach 75%: the lower limit is 74%.

Not validated. Below the threshold.

Rules: reported coverage is validated if it lies within the 95% CI. If surveyed coverage is less than 10 points above 75%, judge the threshold by the lower limit.

Sources: WHO field manual (2019), section 3.1 and table 1.3. Illustration only: not survey data.

Training the teams: three days

Day 1 | Classroom

- Purpose and design
- Segmentation and household selection, with exercises
- Counting and listing children (H5)
- Household form v0.7, F4; consent and assent
- Interviewing; the digital form; role play

Day 2 | Field

- Villages and wards outside the sample
- Small groups led by master trainers
- Segmentation, Lists A and B, interviews on paper and devices
- Debrief; common errors
- Recounting sample registers (F10)

Day 3 | Classroom

- Quality assurance: F4 sign-off, back-checks (F8), observation (F9)
- Data upload and daily checks
- Field plan and logistics
- Post-test (pass mark 80%)
- Issue of List A/B cards, team QR codes, phones and tablets



Master trainers: each leads a group of two to three teams, observes practice with F9, marks the post-test and repeats practical exercises with anyone below 80% before fieldwork. AIIMS leads the training; WHO can join sessions online on request.

Sources: Technical support package, SOP 7 and table 10; field manuals 1–5 (November 2026).

Phones, codes and practice data

	What	Rule	When
Teams	ratlam01–10, chhindwara01–10, shahdol01–10, khandwa01–10 = teams 1–40	Both phones of a pair scan the same code; household form, F4, F10	Training day
Nodal	district_nodal (+ nodal2 in Chhindwara, Shahdol); aiims_monitor	F8 and F9 only; codes sent privately, never in groups	Training day
Practice	Dummy households, one F4 and one F10 per pair, on the live codes	Send them; leave no drafts on the phone	Training day
Pause	No entries of any kind	Phones stay set up; forms update by themselves	Diwali break
Clean-up	All practice submissions backed up, then deleted by AIIMS	Report checked again on day 1	Day before survey
Analysis	Only forms started on or after the first survey day	Written into the analysis plan	Always

Sources: ODK Central project NDD-CES-MP; scripts create_app_users.py, deploy_forms.py and delete_practice.py (AIIMS Bhopal, November 2026).

Practical exercises and materials

Exercise	Materials	When
PPES selection on the Training sheet	Laptop with the workbook	Day 1
Segmenting a sketch map	Printed sketch maps; F4	Day 1
Role plays with F5–F6	Forms; consent statements; sample tablet	Day 1
Digital form with dummy households	Devices with the form installed	Days 1–2
Walking a segment with List A or B	Chalk; laminated lists; a coin	Field day
Recounting a register	Sample school and AWC registers; F10	Field day
Back-check and observation	F8; F9	Day 3

Scenarios for discussion

1. The village has 60 households, not the 150 expected. How many segments?
2. The selected house is locked; the family is away at a wedding for a week.
3. A 12-year-old says he was given the tablet to take home and threw it away.
4. The headteacher has no copy of the reporting form.

Sources: Technical support package, SOPs 2–9; workbook, Training sheet.

Ten rules for the field

1. Never change the number of segments or parts
2. Draw the segment and the list at random, in front of the guide
3. Visit every household on the list, to the end of the segment
4. Never replace a household or a cluster
5. Count all residents; list every child aged 1–19 who lived there during 6–9 October
6. Show the tablet; never suggest an answer
7. Interview without teachers, AWWs or ASHAs present
8. Count only children who swallowed the tablet
9. Recount registers twice, independently
10. Send data every evening; never change data to make it agree

Sources: Technical support package, SOPs 3, 5, 8 and 9.

Post-test (1 of 2)

1. What is the primary indicator of the survey?

a) Children offered the tablet ÷ children surveyed
b) Tablets distributed ÷ target population
c) Children who swallowed the tablet ÷ children surveyed
d) Children ticked in registers ÷ children enrolled

2. A subunit was planned with 3 segments (131 households); the team finds 106. How many segments?

a) 2
b) 3
c) Ask the supervisor to recalculate
d) Enough to keep 50 households in each

3. Which households are interviewed in a selected segment?

a) The first 38 from the starting point
b) Those chosen by the local guide
c) Every household
d) Those on the list chosen by the coin toss

4. A selected house is locked; the family returns in the evening. What does the team do?

a) Return later the same day; if still absent, record it; no replacement
b) Interview the next household instead
c) Ask a neighbour to answer
d) Skip it without recording

5. Who answers for a child aged 7 years?

a) The child
b) A parent or caregiver
c) The class teacher
d) Any household member aged 10 or above

6. Who is listed by name in H5?

a) Children present at the visit
b) Every usual resident, of all ages
c) Every child aged 1–19 who lived there during 6–9 October, absent or not
d) Children enrolled in school

7. Meena (19) moved to Indore after the round. What happens?

a) She is not listed
b) She is listed and a household member answers as proxy
c) The team waits until she returns
d) She is listed but not interviewed

8. H5b says 3 children, but only 2 are listed. What does the form do?

a) Accepts the form
b) Adds a third child automatically
c) Stops until the count or the listing is corrected
d) Flags the household for back-check

Post-test (2 of 2)

9. At a school the recount is 180 and the reported figure is 200. The VF is:

a) 0.90: within the acceptable range
b) 1.11: under-reporting
c) 0.80: over-reporting
d) 20 children

10. Reported coverage is 97%; surveyed coverage is 88% (95% CI 84–92%). Conclusion?

a) Reported coverage is validated
b) Coverage is below the 75% threshold
c) Not validated: reported coverage is above the upper limit
d) The survey must be repeated

11. Surveyed coverage is 80% (95% CI 74–86%). Against the WHO threshold of 75%:

a) Clearly above the threshold
b) Not shown to reach it: the lower limit is below 75%
c) Below the threshold
d) The threshold does not apply to NDD

12. When may a cluster be replaced?

a) When it is far from the road
b) When the sarpanch is not available
c) Never by the team: record it and try again; AIIMS and the coordinator decide
d) When fewer than 38 households are found

13. What does the supervisor do with households flagged for back-check?

a) Replace them
b) Re-interview them within 48 hours with F8 and compare
c) Send them to WHO
d) Delete their data

14. Who must not accompany the team to households?

a) Teachers, AWWs and ASHAs who gave the tablets
b) The local guide
c) The sampler
d) The supervisor

15. Where is record verification done in each cluster?

a) At every school in the block
b) At the nearest health facility
c) The government school most sampled children attend, and the AWC
d) Only at the district office

Pass mark: 12 of 15 (80%). Below the mark: repeat the practical exercises and retake before fieldwork.

References

1. World Health Organization. Preventive chemotherapy: tools for improving the quality of reported data and information. A field manual for implementation. Geneva: WHO; 2019 (https://iris.who.int/handle/10665/329376).

2. WHO Country Office for India. Independent coverage validation of the National Deworming Day round in Madhya Pradesh, October 2026: recommended methodology. Technical note, 29 September 2026.

3. WHO Country Office for India. Independent coverage evaluation survey of the NDD round, Madhya Pradesh, October 2026: technical support package, version 1.0, 6 October 2026.

4. Ministry of Health and Family Welfare. National Deworming Day: operational guidelines. New Delhi: Government of India; 2016.

5. Ministry of Health and Family Welfare. Anemia Mukt Bharat: intensified National Iron Plus Initiative. Operational guidelines for programme managers. New Delhi: Government of India; 2018.

6. Anemia Mukt Bharat training tool kit: deworming schedule by target group, p. 30 (https://www.i4n.in/wp-content/uploads/2023/05/Anemia-Mukt-Bharat-Training-Modules-english.pdf).

7. International Institute for Population Sciences, ICF. National Family Health Survey (NFHS-5), India, 2019–21: Madhya Pradesh. Mumbai: IIPS; 2021.

8. Evidence Action, Deworm the World Initiative. Process monitoring and coverage validation of schools and anganwadis based National Deworming Day in Uttar Pradesh: report; 2016.

9. Zoerhoff KL, Mbabazi PS, Gass K, et al. How well do coverage surveys and programmatically reported mass drug administration coverage match? BMJ Glob Health. 2023;8(5):e011193.

10. Budge PJ, Sognikin E, Akosa A, et al. Accuracy of coverage survey recall following an integrated mass drug administration for lymphatic filariasis, schistosomiasis, and soil-transmitted helminthiasis. PLoS Negl Trop Dis. 2016;10(1):e0004358.